TASK 2 – ADR REPORTING & DOCUMENTATION

VirtualWorks Lab | Pharmacovigilance Internship | Task ID: task2

1. Objective

To document a suspected adverse drug reaction using a structured pharmacovigilance reporting format, following the field structure of the Pharmacovigilance Programme of India (PvPI) suspected ADR reporting form.

2. Case summary

Field	Case information
Patient initials	R.P. (full name in tutorial: Ramesh Patel)
Age / gender	32 years / Male
Weight	65 kg
Diagnosis / indication	Acute bacterial sinusitis
Relevant history	No known history of allergy, as stated in the tutorial
Family history	No history of diabetes or hypertension, as stated in the tutorial

3. Suspected medicine details

Field	Entry
Suspected medicine	Amoxicillin
Dose	500 mg
Route	Oral
Frequency	Three times daily (TID)
Prescribed duration	5 days
Therapy start date	01/03/2026
Therapy stop date	03/03/2026
Indication	Acute bacterial sinusitis
Manufacturer / batch / expiry	Not provided
Concomitant medicines	None reported in the tutorial

4. Suspected adverse reaction details

Field	Entry
Reaction	Generalized itching and skin rashes over the chest
Onset date	03/03/2026
Date medicine stopped	03/03/2026
Management	Amoxicillin discontinued; antihistamines administered
Recovery date	06/03/2026
Outcome	Recovered completely
Seriousness	Non-serious, as described in the tutorial scenario
Relevant investigations	Not provided

Field	Entry
Rechallenge	Not reported

5. Reporter information

Reporter name, designation, professional address, contact number, email and signature: to be completed only if the training form requires them. Use accurate details and do not invent professional credentials or contact information.

6. Causality considerations

Temporal relationship: The reaction began two days after amoxicillin was initiated, which is temporally compatible with a drug-related reaction.

Dechallenge: The patient recovered after amoxicillin was discontinued. Antihistamines were also administered, so recovery after withdrawal alone does not prove causality.

Alternative causes: No concomitant medicines were reported, but other possible causes have not been fully evaluated.

Rechallenge: Not reported. Rechallenge should not be performed solely to confirm an ADR.

Causality interpretation: A possible association is supported by the available information; a definitive category cannot be established without further clinical details.

7. Medical case narrative

A 32-year-old male patient weighing 65 kg was prescribed oral amoxicillin 500 mg three times daily for acute bacterial sinusitis, beginning on 1 March 2026, for a planned duration of five days. On 3 March 2026, the patient developed generalized itching and skin rashes over the chest. Amoxicillin was discontinued on the same day, and antihistamines were administered to manage the reaction. The patient recovered completely by 6 March 2026. The tutorial describes the event as non-serious. No other concomitant medicines were reported. Rechallenge information and relevant laboratory investigations were not provided.

8. Conclusion

The case describes a suspected amoxicillin-associated skin reaction. The timing of onset and recovery after discontinuation support a possible association. The case has been documented using a structured PvPI-style format, with unavailable information clearly identified. Further information would be required for a complete clinical assessment and validation of all ICSR minimum criteria.

9. Reference

Indian Pharmacopoeia Commission. Pharmacovigilance Programme of India (PvPI), Suspected Adverse Drug Reaction Reporting Form for Healthcare Professionals. Official PvPI resources.

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